Five-Person Business Pitch Script

12-slide deck • Suggested length 10–12 minutes including live demo • Assign the five names before rehearsal

PRESENTER 1 | Problem and audience

Assigned speaker: [MEMBER 1] • Coverage: Slides 1–3

Team responsibility: Business problem, main user, beneficiary and potential buyer

Good afternoon. We are the ResearchVoice AI team. Our goal is straightforward: help clinical researchers capture study data faster, without giving an AI system the authority to make clinical decisions.

Think about a researcher running a study with repeated patient visits. They collect observations from an interview or notes, move to a spreadsheet, type each field, check whether anything was missed, and often return to correct something. That repeated work is the problem we are trying to solve.

Our first user is a clinical researcher or physician-investigator. The beneficiaries also include research coordinators and data managers. The potential buyer might be a principal investigator or a research department, but we have not yet confirmed the budget owner.

We're not claiming measured time savings yet. Our business plan uses a planning estimate of eight minutes per visit, and our next pilot will measure the real baseline. I'll hand over to [MEMBER 2], who will show how we structured the solution.

Cue card / handoff

Lead with the problem. Baseline is an estimate, not measured. Hand off to architecture.

PRESENTER 2 | Architecture and components

Assigned speaker: [MEMBER 2] • Coverage: Slides 4–5

Team responsibility: One workflow, five component owners, safety at handoffs

ResearchVoice AI is one integrated n8n system, with five functional components owned by our five team members. Our instructor approved this integrated arrangement instead of separate workflow files.

The first component lets a researcher define and approve the study variables. The second receives text or voice through Telegram and checks for risky content. The third maps the confirmed transcript to the approved variables, asking for evidence rather than making up values.

The fourth component runs an independent AI quality judge, records a LangSmith QA checkpoint, and issues monitoring alerts. The fifth component puts the record back in front of the researcher: they correct any unclear field, approve it, and only then receive the Excel file.

The main design rule is that missing information remains missing until the researcher corrects it. The system is assistive, not autonomous. [MEMBER 3] will demonstrate the workflow.

Cue card / handoff

Name five functional components; instructor approved one n8n workflow. Hand off to live demo.

PRESENTER 3 | Live Telegram demonstration

Assigned speaker: [MEMBER 3] • Coverage: Slide 6 + live app

Team responsibility: Demonstrate typed synthetic path; use voice only if already verified

I'll show the workflow from the researcher's perspective, without going through the code. We have an approved demo study with two variables: Age and HbA1c. I will select that study, start a fictional case, and enter a visit date.

For this synthetic visit, the description is: 'The patient is 54 years old and HbA1c is 6.2 percent.' The assistant first asks us to confirm the transcript. After confirmation, it produces a structured review using only our two approved fields.

The important part is here: we check the values. If a field is unclear, we correct it or explicitly accept it as missing. We then press Confirm & Export. The bot sends an XLSX file only after researcher approval.

We can also open /eval and /monitor to see the quality-check summary. The demo uses fictional data; no real patient information is sent. I'll pass to [MEMBER 4] for the measured demonstration results.

Cue card / handoff

Confirm transcription → review → human approval → XLSX. Never show real data; have backup video ready.

PRESENTER 4 | Quality, results and monitoring

Assigned speaker: [MEMBER 4] • Coverage: Slides 7–8

Team responsibility: One observed synthetic scenario; AI scores are not accuracy claims

We have evidence from an actual synthetic Phase 4 test. The voice transcription altered the HbA1c variable name. Instead of pretending the field matched, ResearchVoice flagged it and sent a soft alert asking for review.

The researcher corrected the value manually and approved the export. The XLSX arrived through Telegram. In that documented test, the independent judge gave an average of 4.6 out of 5: five for factual accuracy, groundedness, safety and bias, but three for format adherence because the field remained unresolved in the AI draft.

The judge stage took approximately 10.49 seconds in that snapshot. LangSmith successfully accepted the QA trace, and the /monitor command displayed the warning and export count.

These figures describe one synthetic demonstration, not a clinical accuracy study or a statistically reliable benchmark. Independent human scoring is still required to calibrate the judge. [MEMBER 5] will finish with the business value, safety limits and proposed pilot.

Cue card / handoff

Cite exactly one observed snapshot (4.6/5, 10.49 s, soft alert, one XLSX). Emphasize uncalibrated scores.

PRESENTER 5 | Business value, safety and pilot ask

Assigned speaker: [MEMBER 5] • Coverage: Slides 9–12

Team responsibility: Proposed KPIs, limitations, realistic next step and Q&A

Our business hypothesis is that assisted capture will reduce repeated manual work and make unclear fields easier to identify. We proposed targets for preparation time, exact field agreement, corrections, successful exports, and ease of use. The baseline and real actuals are still to be measured.

We have already implemented schema restrictions, a researcher confirmation step, safety checks, and a quality-review alert. But the prototype is not ready for real patient data. The privacy gate is best-effort, voice reaches the transcription provider before we can screen its words, and the current storage is not suitable for production healthcare use.

Our request is a controlled synthetic-data pilot, access to independent reviewers, and feedback from a potential research-unit buyer. We previously proposed 20 discovery interviews, around 10 interested pilot users and two research teams as targets, not current commitments.

After measuring those results and completing governance review, we can decide if a secure real-world pilot is justified. Thank you. We are happy to answer questions about the research workflow, safety, results and potential value.

Cue card / handoff

State targets as targets, pilot conditions, no real-data use. Invite questions.